
CONSULT FOR AFRICA

Osteon Clinics

Organisational Audit: Information and Data Request

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For Dr Bolarinwa Akinola, FRCS (Tr. and Orth.), and the Osteon Clinics leadership

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Lagos and Abuja, Nigeria



A note before the list

Dr Bola, I have been thinking about Osteon since we finished the positioning work in July, and the conclusion I keep coming back to is that we cannot build the brand any further until we know what the business behind it can actually carry. The positioning we agreed asks you to be the surgeon other surgeons refer to, and it asks Osteon to stand behind that as a named centre of excellence, which is a promise about theatre discipline, sterile processing, implant traceability and outcomes as much as it is a promise about surgical skill. So this audit looks at the whole organisation, end to end, rather than at the parts that face the patient.

I want to say two things plainly up front.

The first is that nothing in this list is a test. Where a record does not exist, tell us it does not exist, because the absence is itself a finding and it is usually a more useful one than a tidy file would have been. We would far rather see the real spreadsheet, the WhatsApp group where cases get booked and the notebook at the front desk than a version reconstructed for us. Reconstruction wastes your team's week and it tells us nothing about how the place runs on an ordinary Tuesday.

The second is that this is confidential and it is ours alone. Your numbers, your case log and your staff's answers do not leave Consult for Africa, they are not shared with any hospital you visit, and they are not shared with OrthoSurplus or any other party unless you tell us in writing to share them.

We will be on site on Thursday 17 September. The priority list on the next page is what we need before that day, because it is what turns a tour into an audit. Everything else can follow over the two weeks after.

Warm regards,

Dr Debo Odulana

How to send it

Please nominate one point of contact who can chase the rest of the team, and create a single shared folder (Google Drive or OneDrive) with a sub-folder per lettered section below. Exports from a system are better than typed summaries, so a raw CSV or Excel export from the billing software, the accounting package or the appointment book is preferred over anything anyone has to retype. Photographs of paper registers are fine. Where something is commercially or personally sensitive, mark it and send it anyway, because it is handled under the confidentiality above.

Where a question is easier to answer in conversation than in writing, write "discuss" and we will cover it on Thursday.

Before Thursday: the nine that matter most

These nine unlock everything else, and they are the ones we will be working from while we are with you. If the week runs away, send these and nothing else.

#	What we need	Why it is first
1	The surgical case log for the last 24 months, every case, every site	It is the spine of the audit. Everything else is checked against it
2	Management accounts or the income and expenditure record, monthly, last 12 months	Tells us what the business earns and what it costs to stand still



#	What we need	Why it is first
3	The current price list or tariff, every service, and how a surgical quote is built	We cannot judge yield without knowing the list
4	Full staff list with role, employment type and days worked per week	Establishment against service is where small clinics fail
5	Implant and consumable purchase records, last 12 months, with unit prices	Implants are the largest single cost in your bill
6	Bank statements for all business accounts, last 6 months	The only unarguable record of what actually came in
7	Facility registration and licence documents, whatever is held	Category registered against service delivered is a live exposure
8	The agreements, or a description, of your arrangements at Cedarcrest, Diamed and elsewhere	Most of the practice may sit outside the company
9	Theatre list schedule or diary for the last 6 months	Utilisation is the profit driver and nobody measures it

A. The company, ownership and how decisions get made

- Certificate of incorporation, memorandum and articles, and the CAC status report
- Current shareholding, and whether any shares are held by a non-Nigerian person or entity
- Directors, and any shareholders' or partnership agreement
- Where Osteon Clinics sits in relation to your personal practice: is your consulting and operating income invoiced through the company, through you personally, or both
- Any other companies you own or control that touch this work, including any equipment, property or supply entity
- How the business is governed today: who meets, how often, what is minuted, and who signs off spending above a threshold
- Any board, advisory or management meeting notes from the last 12 months
- Any loans, overdrafts, equipment finance, related party balances or personal money put into the business, with terms

B. Licensing, regulatory standing and insurance

- Facility registration with HEFAMAA, including the certificate, the category the premises is registered under and the current expiry date
- Any correspondence with HEFAMAA, including inspection reports and any conditions or undertakings given
- The named medical director of record for the premises, and confirmation that the certificate is not being used for any other facility
- MDCN full registration and current annual practising licence for every doctor who works here, including visiting and locum surgeons and anaesthetists
- Nursing and Midwifery Council licences for every nurse, including theatre and recovery staff



- Radiation: the Nigerian Nuclear Regulatory Authority licence for any X-ray unit, image intensifier or C-arm, the radiation protection officer's appointment, the last equipment quality assurance report, and personal dosimetry records for staff who work near the beam
- Pharmacists Council premises licence and superintendent pharmacist, if any medicine is dispensed or sold from the site
- Medical Laboratory Science Council registration, if any laboratory work is done on site
- NAFDAC position on the implants and devices you use: who imports them, under whose registration, and evidence of registration for each implant system
- Professional indemnity: the surgeon's cover, the facility's cover, and cover for anaesthetists and nurses, with sums insured and expiry
- Public liability, fire, burglary and equipment insurance
- Clinical and sharps waste: the contract, the carrier, and the consignment notes
- Fire safety certificate, environmental permit, and any local government or signage permits
- Any current or threatened claim, complaint to a regulator, or litigation, and any patient complaint that escalated beyond the clinic
- Data protection: whether the business is registered with the Nigeria Data Protection Commission, who the data protection officer is, and what patients are told about their records

C. The practice across sites, which is the boundary question

This section matters more than its length suggests. You operate at Osteon and you operate elsewhere, and we need to see the whole practice before we can say what Osteon itself is worth.

- A list of every site you consult or operate at, with days per month at each over the last 12 months
- The written agreement, or a description of the understanding, at each: Cedarcrest, Diamed and any other
- How you are paid at each site: fee per case, session fee, percentage of the bill, or direct billing of the patient
- Who bills the patient at each site, who holds the patient relationship afterwards, and who owns the record
- Revenue earned at each site over the last 12 months, and whether it is invoiced through Osteon Clinics or to you personally
- What Osteon provides to a case done elsewhere, if anything: instruments, implants, staff, follow-up, physiotherapy
- What proportion of the cases you do elsewhere could have been done at Osteon, and the reason each was not
- Any restriction on you, formal or informal, on taking a case from one of those hospitals to Osteon

D. Money: what comes in and what it costs to stand still

- Monthly management accounts, profit and loss, for the last 24 months if they exist, 12 at minimum
- If formal accounts are not prepared, the income and expenditure record in whatever form it is kept
- Latest balance sheet, trial balance and chart of accounts
- Bank statements for every business account, last 12 months, and confirmation of any personal account used for business receipts
- The accounting system in use, and read access or a full export



- Any audited or accountant prepared financial statements, and the most recent tax filings including PAYE, VAT and company income tax status
- Fixed asset register with purchase dates and values
- Monthly fixed cost run rate: rent or mortgage, salaries, diesel and power, water, internet, security, cleaning, software, insurance
- Current budget or plan for this year, if one exists, and actuals against it
- Cash position today, and the largest single month of loss in the last two years with the explanation

E. The surgical ledger: case mix, volume and what each case earned

This is the single most important dataset in the audit. One row per case, for the last 24 months, across every site. Whatever fields exist is fine; do not create fields that do not.

- Date of surgery, site, procedure, side, primary or revision, elective or trauma
- Patient age and sex, no names needed for the analysis although we will need identifiers on site to trace a sample
- Surgeon, assistant, anaesthetist
- Implant system and components used, with catalogue and lot or serial numbers
- Payer: self pay, family abroad, HMO and which one, corporate, or charity
- Amount quoted, amount invoiced, amount actually collected, and any discount or write off with the reason
- Implant cost for that case, and any other consumable cost captured
- Length of stay, and whether the patient was discharged as planned
- Any complication, readmission, return to theatre, or infection, and the follow up interval reached
- Separately: the same ledger for non-surgical activity, meaning clinic attendances, injections, casting, dressings, imaging and physiotherapy sessions, monthly by type

F. The funnel: how many people ask, and how many end up on the table

Elective surgery is a long decision, and most practices lose most patients somewhere in the middle without ever knowing where. If none of this is tracked, say so, and we will start tracking it from Thursday.

- Enquiry log in any form: phone, WhatsApp, website, walk in, Instagram, monthly for the last 12 months
- New patient consultations per month, and how many were new versus follow up
- Of new consultations, how many were advised surgery, how many were quoted, how many paid a deposit, and how many were operated on
- Typical elapsed time from first consultation to surgery
- Reasons patients give for not proceeding, in whatever form they are recorded or remembered
- Any patient who went abroad instead, and what you know about why
- Cancellations and postponements, with the reason, and who cancelled
- The WhatsApp or phone number enquiries land on, who answers it, and what happens to an enquiry that arrives at nine at night

G. Who pays, and how well



- Current published or quoted price for each of: primary total knee, primary total hip, revision knee, revision hip, infected joint two stage, major trauma fixation, arthroscopy, consultation, and follow up
- How a quote is built: which components are itemised, what is included, what is excluded, and who may vary it
- Any package or fixed price offer currently in use, and what it covers
- Split of the last 12 months by payer type, by value and by case count
- Every HMO or insurer contract and tariff schedule held, by the clinic or by you personally
- HMO claims register: submitted, approved, rejected, paid, outstanding, with dates, and the reasons for rejection
- Debtors ageing today, by payer, with the oldest item
- Corporate or employer accounts, and their terms
- Deposit and credit policy, written or in practice, and who may waive it
- How a family abroad pays: the mechanism, the currency, who handles it, the fees lost, and how long it takes
- Any instalment, financing or payment plan arrangement, formal or informal, and how many patients have used it

H. Theatre and the perioperative pathway

- Theatre list schedule or operating diary for the last 6 months, showing planned lists and what actually ran
- Number of theatres, table time available per week, and the days theatre is genuinely usable
- Planned start time against actual start time, and actual finish, for as many lists as you have record of
- Turnaround time between cases, and how it is measured if at all
- Cancellations on the day of surgery, with the reason for each
- Pre-operative assessment: who does it, when, what is checked, and what would stop a case going ahead
- The anaesthetic service: employed, visiting or freelance, who covers, how they are paid, and what happens if they are unavailable
- Post-anaesthetic recovery: where, staffed by whom, monitored how, and for how long
- The WHO surgical safety checklist or equivalent: whether it is used, and a sample of completed ones
- Antibiotic prophylaxis policy and timing, and venous thromboembolism prophylaxis policy
- Escalation and transfer: the written arrangement with a hospital that has intensive care, the ambulance arrangement, and the last time either was used
- Blood: how a transfusion is arranged, from where, and how long it takes
- Inpatient beds on site, staffing overnight, and who reviews a patient at two in the morning
- Post-operative ward round routine, discharge criteria, and the discharge pack a patient leaves with

I. Implants, instruments and consumables

Implants are the largest line in your cost of sale and the most regulated thing you put in a patient, so this section carries both the money and the risk.

- Every implant supplier used, with the commercial basis: outright purchase, consignment, per case invoice, or rep delivered on the day



- Purchase records and supplier invoices for the last 12 months, with unit prices per component
- Current implant stock on site, by system, size and lot number, with value
- The traceability record: how the lot or serial number of every implanted component is recorded against the patient, and where that record lives
- The recall process: if a manufacturer recalled a batch tomorrow, how would you identify the patients
- Who chooses the implant for a given case, on what basis, and whether any supplier arrangement influences it
- Loan kit process: how a set is ordered, when it arrives, who checks it is complete, who sterilises it, and what happens when it arrives late or short
- Whether implant company representatives attend theatre, what they do there, and what governs that
- Other high value consumables: cement, cement mixing systems, drapes, gowns, sutures, tourniquets, drains, dressings, with usage and cost per case
- Any stock out in the last 12 months that delayed or cancelled a case
- Drugs held on site: the list, who orders, how they are stored, expiry management, and the last physical count
- Controlled drugs: what is held, the register, the cupboard, and who holds the key

J. Sterile services and infection prevention

For arthroplasty this is the difference between a good outcome and a catastrophe, so we will look at it closely and without any assumption that a small clinic must be doing it badly.

- Where instruments are decontaminated and sterilised: on site, or sent out, and to whom
- If on site: the autoclave make, model, age, chamber size, service history and validation certificates
- Cycle records for the last 3 months, and the daily test records, meaning the air removal test and the chemical and biological indicators
- The load record: how a specific sterilised set is traced to the patient it was used on
- Single use items: confirmation that nothing single use is reprocessed, and the policy that says so
- Theatre air: the ventilation arrangement, whether there is laminar flow, filter change records and any air sampling ever done
- Theatre discipline: who is allowed in, movement during a case, door openings, footwear and clothing policy
- Cleaning: the between case routine, the end of list routine, and the deep clean schedule, with records
- Surgical site infection: every case in the last 24 months, how it was defined, how it was treated, and where it is recorded
- Hand hygiene, sharps handling, and any staff needle stick incident
- Any infection prevention policy document, and any audit of it

K. Clinical governance, outcomes and safety

- Patient records: paper, electronic or both, where they are kept, who can access them and whether they are backed up
- A blank set of the forms in use: clerking, operation note, consent, anaesthetic chart, drug chart, nursing observations, discharge summary
- Consent: the form used, who takes it, when, and what risks are written down



- Incident, near miss and complication register, in whatever form it exists
- Mortality and morbidity review: whether it happens, who attends, and any notes
- Outcome measurement: whether any score is collected before and after surgery, such as the Oxford Hip or Knee Score, range of movement, pain score or return to work, and at what intervals
- Follow up: the schedule, what proportion of patients actually attend each interval, and how patients who stop attending are chased
- Revision and re operation: every case in the last 24 months where a patient came back to theatre, and why
- Readmissions within 90 days
- Patient complaints, and what happened to each
- Any clinical protocol, guideline or standard operating procedure that exists in writing, and the date it was last reviewed
- Any audit the clinic has ever done of its own practice
- Testimonials, before and after imaging, and any patient consent held for using their story

L. Imaging and diagnostics

- What imaging is available on site: plain film, image intensifier or C-arm, ultrasound
- Make, age, service record and current working state of each
- Who reports the images, and how quickly
- Where a patient goes for CT, MRI or bone scan, the arrangement with that provider, the cost to the patient and the turnaround
- Laboratory: what is done on site, what is sent out, to whom, at what price, and the turnaround
- Whether any commission, referral fee or margin is earned on imaging or laboratory work sent out

M. Physiotherapy and recovery

- Whether physiotherapy is provided on site, by employed, sessional or visiting therapists
- The post-operative rehabilitation protocol for a knee and for a hip, if written
- Sessions delivered per month, and what is charged for them
- What proportion of surgical patients complete the prescribed course
- What happens to a patient who lives far away, and whether anything is done remotely
- Equipment available for rehabilitation, and the space it uses

N. People

- Full staff list: name, role, department, employment type, start date, days worked per week, and whether they work anywhere else
- Payroll for the last 3 months: gross, allowances, deductions, net, and how and when people are actually paid
- Employment contracts or offer letters, a sample plus every clinical role
- Job descriptions, if they exist
- The rota for the last 2 months, including nights, weekends and theatre days
- Any locum, sessional or visiting staff, how they are found, and what they are paid
- Joiners and leavers in the last 24 months, with the reason for each departure



- Training records: resuscitation, theatre practice, sterile technique, infection control, and any continuing professional development
- Statutory position on PAYE, pension and any employee benefit
- Any disciplinary or grievance matter, and any staff complaint
- Who runs the clinic day to day when you are operating, and what they are and are not allowed to decide
- What the team is told about the plan for the business, and how

O. The building, the equipment and the power

- The premises: owned or leased, the lease terms and expiry, the landlord, and the rent
- A floor plan, or a sketch, with room names and areas
- Rooms and their use: consulting, theatre, recovery, ward, sterile services, store, imaging, physiotherapy, office, reception, waiting, staff, toilets
- Bed numbers, and how many are ever occupied at once
- Parking, access, and how a patient on crutches gets from the car to the consulting room
- Asset register, or a walk through list, of every significant piece of equipment with its working state
- Power: grid supply, generator make and size, hours run per day, diesel cost per month, inverter or solar, and the last time theatre lost power mid case
- Medical gases: oxygen supply, cylinders or concentrator, suction, and who checks them
- Water: source, storage, treatment, and what happens when it fails
- Any planned or deferred maintenance, and anything currently broken

P. Systems, records and reporting

- Every piece of software in use: appointments, clinical records, billing, accounting, messaging, imaging storage
- Who administers each, who has access, and whether access is by individual login
- Backups: what, where, how often, and when a restore was last tested
- The management report or dashboard, if any, that tells you how the business is doing, and how often you look at it
- The numbers you personally watch, and the number you would want to see every Monday morning if you could have only one
- How a patient's record moves between Osteon and the hospitals you visit

Q. Brand, referral and demand

- Website analytics for the last 12 months, and who controls the site
- Social accounts, follower counts, and who posts
- Every marketing spend of the last 24 months, and what it produced
- Referral log: which doctors, physiotherapists and clinics have sent you a patient in the last 24 months, and how many each
- What a referring doctor receives back after they refer, and how long it takes
- Any teaching, meeting, case discussion or event you run or attend that puts you in front of referrers
- Enquiries from abroad, and what happens to them



- Anything you have tried to grow the practice that did not work

R. Where you want to take it

- What you want Osteon to be in five years, and whether it is your practice vehicle or an institution that outlives you
- The single constraint you would remove tomorrow if you could remove only one
- Your view on bringing in associate surgeons, and what would have to be true first
- Your appetite for a second site, and where
- Any conversation already under way with a hospital, investor, supplier or partner that we should know about
- What a good outcome from this audit looks like to you, in one sentence

What we do with all of it

We read it before Thursday, so that the visit is spent looking at the things the documents cannot show us: the theatre, the sterile room, the implant store, the way a list actually runs, and how your team talks about the work. On the day we will also trace ten real cases end to end, from the first enquiry through consent, theatre, the implant sticker, the sterilisation record, the invoice, the payment and the follow up, because tracing a small number of cases completely tells us more than sampling a large number of documents partially. We will tell you which ten on the morning.

Four short surveys go out alongside this request, to your staff, to your patients, to the doctors who refer to you, and to you and your leadership team. They take five to ten minutes each and the staff and patient ones are anonymous. The referrer survey is the one I care most about, because the referral pipeline is the constraint on this practice and we have never actually asked those doctors what they want.

Two weeks after the visit you get the audit report: what we found, what it costs you, what to fix first, and what the business can carry. Then we decide together what we build.

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